



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : سهره ابراهيم رزق غطاس
المحافظة : القاهرة
الحى / المدينة : المرج
رقم البطاقة : 27608142501066 رقم التليفون : 01200547724
العنوان : 27608142501066
العمر (تاريخ الميلاد) : 14-08-1976 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 20-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مسح ذري علي الجسم [8530] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.
تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- / د. / ----- / د. / ----- / د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: سهره ابراهيم رزق غطاس National ID: 27608142501066	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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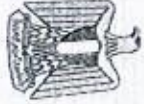
المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Whole-body PET/CT scan

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

DMS

← كارت

٢٠٢٤/٠٥ ٢٧٦٠٨١٤٢٥٠١٠٦٦
حصلته على بطون زراعه
أنش مسجبة
البطاقة سارية حتى ٢٠٣١/٠٥/٢٩



جمهورية فلسطين العربية
مجلس تحقيق الشخصية

سجله
ابراهيم زريق غطاس
أنش الراعي الصالح - صبرين الخطيب
المرج - القنود



K03443105 ٢٧٦٠٨١٤٢٥٠١٠٦٦



SCAN & LABS

مركز النيل للأشعة والتحاليل

Name : Sahara Ibrahim Rizk Ghattas
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 04-02-2026 10:47
Medical No. : 309493
Ins.Company : تعاقد مستشفيات

Tc^{99m} MDPSKELETAL SCINTIGRAPHY

History

- A 49-year-old lady, with history of left breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings

- No abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,

Dr. Alaa Nagi, MD
Nuclear Medicine

Call Center 19656

Name : SAHRA IBRAHEM REZK

Date : 08-02-2026 09:55

Branch : Portable

Medical No. : GRI-27608142501066

Ref.Physician Prof : CT CAP +C

Ins.Company :

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

Reported case of left breast cancer

I- CHEST:

- Left breast heterogeneously enhancing mass measuring about 5.4x3cm associated with multiple pathologically enlarged axillary lymph nodes, largest one measuring about 3.2 x2.4 cm
- Bilateral multiple scattered solid pulmonary nodules, largest one seen at left upper lobe measuring about 8mm
- No pathologically enlarged hilar or mediastinal lymph nodes.
- No plural or pericardial collection.
- No gross cardiomegaly
- Dorsal spondylosis.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Small hiatus hernia
- Normal C.T appearance of the spleen, pancreas, both kidneys, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No pathologically enlarged abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- Left breast heterogeneously enhancing mass associated with multiple pathologically enlarged axillary lymph nodes
- Bilateral multiple scattered solid pulmonary nodules? Metastases
- Small hiatus hernia

Dr. Nourhan Mohamed,MD

Dr. Nourhan Mohamed,MD

03/09/2026

MDT FORM

Name:	Sahra Ibrahim Rizk Ghtass	MRN:	8931
Age:	50 Y	ID:	DMS >> 8931
Menopausal Sta	postmenopausal	Phone:	01200547724
NRS SCORE:		Marital Status:	unmarried ٥٧٦٠٨١٤٩٥٠١٢٦٦
BMI:		Family History:	Irrelevant
CO Morbidity:	PS1 on phsycological drugs yet pt is oriented	Reason Of MDT:	new case

Clinical Data:

BX:	IDC
ER:	0
PR:	2
HER2:	+3
KI67:	45%

R

L

Sonomammography:

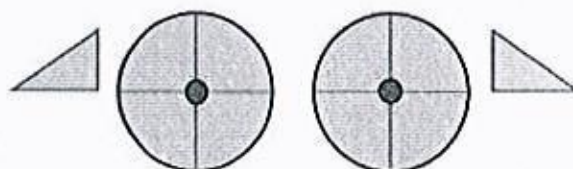
Presented with Lt breast T4b N2 M0

Lt UIQ 9-12 oclock expanding retroareolar 2.3*5.6*3.9 cm

Rt pathological lns largest 2.4 *2 cm

Clinically>> T4b with retracted nipple and very close to the skin lesion

Lns are palpable 2-3 cm



T N M

Mets Workup:

CT CAP>> Free

Bone scan>> free

History:

Recommendations:

MDT Outcome & Plan

TCHP X 6 CYCES THEN RT MRM +/- RECONSTRUCTION

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ نورهان محمد	د/ خالد المعدي				



Women's Health Outreach Program

Name: سهره ابراهيم

Date: 11/11/2025

Full field Bilateral Digital Mammography & ultrasonography

Detailed Findings

Technique

Craniocaudal & mediolateral oblique views

High resolution ultrasound

Left breast

- ACR c.
- Left upper inner dense mass with irregular margin & thickened skin of the nipple.

Right breast

- ACR c.
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.

- Complementary US demonstrated left upper inner heterogenous hypoechoic irregular border solid mass seen at 9: 12 oclock occupaying the whole upper inner quadrant & expanding retroareolar measuring 2.3 x 5.6 x 3.9cm
- Normal skin thickening and nipple areola complex
- Suspected left axillary lymph nodes (rounded heterogeneous with effased fatty hilum at one of them) the largest measures about 2.4 x 2cm.

Conclusion:

-left breast: (BIPADS 5) left upper inner mass as described above with suspected left lymph nodes.

-right breast: normal (BIRADS 1).

Best regards,

Dr. Huda Ahmed

Patient Name : الأستاذ / مبره ابراهيم رزق	Request No. : 13003438
Patient No. : 27608142501066	Request Date : 16/11/2025
Path No. : 3531/25	Verification Date : 25/11/2025
Sex : Female	Financial Category : جوماتاف رومي - مبادره
Age : 49 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data

Left breast mass.

Nature of specimen

Tru cut biopsy collected on 16/11/2025.

Gross examination

Three grey white tissue cores ranging in size from 0.5 cm up to 1 cm, totally submitted .

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against: HER2/NEU (4B5), Ki67 (30-9), PR (1E2), ER (SP1).

Microscopic examination

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (2+2+2) Grade 2
- * DCIS: Absent
- * Lobular intra epithelial neoplasia : Absent
- * Angioinvasion: Absent .
- * Microcalcification : Absent
- * Perineural invasion: Absent
- * IPT (Positive control showed appropriate reactivity) :
- * ER: No nuclear reaction in tumor cells (Allred score 0/8)
- * PR: Weak nuclear reaction in less than 1 % tumor cells (Allred score 2/8)
- * HER2: Strong complete membrane reaction in >10 % of tumor cells (3+)
- * Ki 67: 45 %

Verified by :

Dr.Heba Hasan



Patient Name : الأستاذ / مبره ابراهيم رزق	Request No. : 13003438
Patient No. : 27608142501066	Request Date : 16/11/2025
Path No. : 3531/25	Verification Date : 25/11/2025
Sex : Female	Financial Category : جوسكاف روسي - مبادره
Age : 49 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Diagnosis

LEFT BREAST MASS, TRU CUT BIOPSY

-Invasive duct carcinoma Grade 2 (tubular differentiation 2, pleomorphism 2, mitosis 2)
(according to Nottingham grading system of breast cancer) .

IPT:

- * ER: Negative (score 0/8).
- * PR: Negative (score 2/8).
- * HER2:Positive (score +3)
- * Ki67: 45 %
- * Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

- * Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr.Walaa Ibrahim Ghoneim
Dr.Heba El-Shafeey

Verified by :

• Dr.Heba Hasan

Printing Date : 02/12/2025 10:20:24

Page 2 of 3

Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987-Nile Corniche - Misr El Qadema - Cairo, Egypt

جمهورية مصر العربية
بطاقة تحقيق الشخصية



سمره
البراهيم رزق غطاس
١٢ ايش الراعى الصالح - عشرين القطايب
المرج - القاهرة



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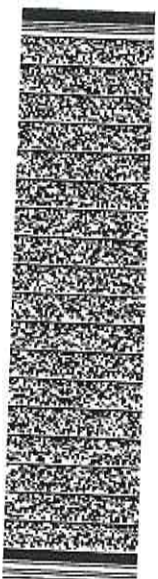
٢٧٦٠٨١٤٢٥٠١٠٦٦
حاصله على ببلوم زراعه



انثى
مسيحية
النسة

البطاقة سارية حتى ٢٠٣١/٠٥/٢٩

١



Patient's Name:
Date:

Sahara Ibrahim Rizk Ghattas
2026-09-13

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Examination:

■ **Computed tomography of the chest, abdomen and pelvis**

Technical data:

- 5 mm CT scans of the chest, abdomen and pelvis with IV contrast

Clinical data: known case of left breast cancer on treatment.

Comparison: 8-2-2026

Findings:

■ Chest findings:

- Left axillary few small non specific LNs
- Regressive size and number of pulmonary nodules currently the largest 5 mm on the left lower lobe compared to 8 mm.
- Clear rest lungs fields with no active parenchymatous infiltrates.
- No hilar or mediastinal masses or sizable LNs.
- Normal appearance of the tracheobronchial tree.
- No evidence of pleural effusion or thickening.
- Average cardiac size, no pericardial effusion.

■ Abdomen and pelvis findings:

- The liver shows average size with smooth contours, homogeneous hepatic parenchyma density and no evidence of focal lesions.
- No intra hepatic biliary dilatation.
- Normal caliber and homogenous enhancement of the portal vein.
- The spleen shows normal size. No evidence of focal lesions.
- Normal CT appearance of the gall bladder, pancreas and adrenal glands
- Both kidneys are of normal site, size and shape, no stones, back pressure, cysts or masses bilaterally.
- The urinary bladder shows average distention, no wall thickening, no stones, diverticulae or masses.
- No suspicious abdominal or pelvis enlarged LNs
- No CT evidence of intestinal obstruction
- No pneumoperitoneum
- No free or localized abdominal or pelvis fluid collection
- Left ovarian 3 cm cyst

Patient's Name:

Sahara Ibrahim Rizk Chattas

Date:

2026-09-13

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Opinion:

- Regressive size and number of pulmonary nodules currently the largest 5 mm on the left lower lobe compared to 8 mm.
- Left ovary 3 cm cyst
- No suspicious abdominal or pelvis metastatic lesions on CT base
- No suspicious bony lesions on CT base.

Recommendation:

- Dedicated breast studies correlation

Regards, Dr. Ahmed Rezk, MD.

Ahmed

Patient's Name:

Sahara Ibrahim Rizk Ghattas

Date:

2026-09-13

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

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Patient's Name:

Date:

Referred by:

Hospital:

Sahara Ibrahim Rizk Ghattas

2026-09-13

Prof.Dr. no doctor

Goustave Roussey International Egypt

Opinion:

- Regressive size and number of pulmonary nodules currently the largest 5 mm on the left lower lobe compared to 8 mm.
 - Left ovary 3 cm cyst
 - No suspicious abdominal or pelvis metastatic lesions on CT base
 - No suspicious bony lesions on CT base
- Recommendation:**
- Dedicated breast studies correlation

Regards, Dr. Ahmed Rezq, MD.

Ahmed